

International Conference on Population and Development at 15 Years: Achieving Sexual and Reproductive Health and Rights for All?

Sexual and reproductive health remains the contentious concept it was at the 1994 United Nations International Conference on Population and Development (ICPD), held in Cairo, Egypt.

In light of the recent 15-year review of ICPD, we suggest several areas where advocates, practitioners, and researchers can inform future progress for sexual and reproductive health. These include the following: improving measurement and accountability related to the evidence base for sexual and reproductive health, indicators of program success, and the tracking of resource flows; creating and renewing alliances to strengthen advocacy; and employing new resource mobilization strategies.

Given the 20-year goals established at ICPD, now is the time to move toward finally achieving the sexual and reproductive health and rights agenda. (*Am J Public Health*. 2010;100:403–406. doi:10.2105/AJPH.2009.177873)

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THE YEAR 2009 MARKED

the 15th anniversary of the United Nations International Conference on Population and Development (ICPD), held in Cairo, Egypt. There is general consensus that ICPD and its Program of Action¹ created a tangible shift from a narrow focus on population and fertility reduction to a broadened agenda that addresses the range of sexual and reproductive health issues that constitute the individual lives of men and women. Sexual and reproductive health and rights were defined to include services and information relating to, for example, family planning, prevention and treatment of HIV and other sexually transmitted diseases, safe abortion, and safe pregnancy—all to be provided in a rights-based approach without coercion, discrimination, or violence.^{2,3}

A goal of universal access to reproductive health for all by 2015 was established, and the progress that governments had made on achieving this and related goals was reviewed by the UN General Assembly in October 2009. Although the earlier 5- and 10-year reviews led to revised actions to strengthen implementation,⁴ preparations for the most recent review were relatively quiet.⁵ There was no new political outcome document outlining revised actions for implementation, regional meetings were unremarkable, and the political rhetoric that had mobilized civil society in the past was muted. This

in part reflected a deliberate decision by the United Nations Population Fund (UNFPA), governments, and civil society supporters of the Program of Action to avoid enflaming any opposition. Indeed, the years since ICPD have not been without their challenges for sexual and reproductive health and rights; in fact, sexual and reproductive health has remained a subject of political, social, and policy debate.^{6,7} The US government had been a major promoter of the Program of Action in 1994 and was its major detractor in 2004. The beginning of the new millennium witnessed a rise in political conservatism, religious fundamentalism, and attention to other global health concerns that have, in some cases, pushed back the ICPD agenda and stalled its progress.

However, the recent political change in the United States with the election of President Barack Obama and the statements from Secretary of State Hilary Clinton⁸ have fueled optimism that sexual and reproductive health and rights issues will be restored to their priority status in US foreign assistance.^{9,10} President Obama's repeal of the Mexico City Policy (also known as the Global Gag Rule), which prevented international organizations from receiving any US government funding to address abortion as part of any of their activities, and the reinstatement of US funding for UNFPA are tangible and encouraging symbols of such change. The future

landscape for sexual and reproductive health and rights may be far from certain. Concern about the impact of the current global economic crisis on health spending¹¹ compounds the already competitive environment for health resources.¹² Nonetheless, we believe the recent 15-year anniversary of ICPD presented an opportunity to be bold and to refocus attention and resources on sexual and reproductive health and rights.

Between 2005 and 2007, a group of scholars associated with the Group on Reproductive Health and Rights at the Harvard Center for Population and Development Studies examined different arguments (e.g., economic, demographic, programmatic, human rights–based) for and against the Program of Action. Taken as a whole, their conclusions demonstrate that the Program of Action speaks directly to the challenges facing global health today: neglect of health systems, increase of poverty and inequity, persistence of gender inequality, and stagnation of health indicators across the board. The results of their work were published in 2009; we offer the following observations based on their findings.¹³

RESILIENCE AND RELEVANCE OF ICPD PRINCIPLES

Although sexual and reproductive health and rights may

have slipped from the position of prominence on the international agenda that it had in 1994,^{9,14} its influence has far from dissipated. The “Cairo paradigm” still has resonance across disciplines, and—countervailing international trends notwithstanding—ICPD remains foundational to the achievement of global health and development goals.

The actual meaning of ICPD and its Program of Action depend on an individual’s discipline and experience. The conceptual underpinnings of ICPD can be—and have successfully been—incorporated into arguments regarding public health, human rights, demographics, development, and empowerment, all of which are relevant to broader health and development debates. This diversity of meaning is a reflection of the strength and wide-reaching impact of ICPD. It also calls attention to the fact that ICPD was not minted overnight but reflects a long and rich conceptual and empirical history fostered by a range of movements—human rights, population, development, and social justice, to name a few.

HIV/AIDS AND SEXUAL AND REPRODUCTIVE HEALTH RIGHTS

In the years since ICPD, HIV/AIDS has come to overshadow attention to other aspects of sexual and reproductive health. ICPD may not have said enough about prevention, treatment, and care for HIV/AIDS, but with more women than men now infected with and affected by HIV/AIDS (often referred to as the “feminization” of the HIV/AIDS epidemic) and the advent of more widespread



Iranian women stand in line at the Masoumeh shrine in Qum to cast their votes in the presidential election on June 12, 2009. Photograph by Kamran Jebreili. Printed with permission of AP Wide World.

access to antiretroviral agents, there is a new urgency to bring the reproductive health and HIV/AIDS advocacy and service provision communities together. That HIV has, in some areas of the world, absorbed much of the available health care resources—both human and financial—is another persistent challenge to ICPD, but the rationale for forging common ground goes beyond tapping into financial resources. Rather, it is a larger issue of building functional health systems, pooling scarce human resources, and financially supporting a range of health interventions that meet the goals and aspirations of both communities.

“GLOBAL HEALTH” AND DEVELOPMENT AGENDAS

As important as the UN Millennium Development Goal

indicators are, they do not effectively distill the profound insights resulting from years of activism and research prior to ICPD. The much-vaunted Millennium Development Goal 5(b) target (Universal Access to Reproductive Health)¹⁵ is measured by proxy indicators that leave out many of the sexual and reproductive health issues and all of the human rights aspects of the Program of Action. It is critical, therefore, to keep the core principles of ICPD salient while designing and evaluating the strategies to achieve the Millennium Development Goals related to sexual and reproductive health and rights.

The research generated by the Group on Reproductive Health and Rights¹³ pointed to the overall enduring value of the Program of Action; in addition, it identified some specific

approaches to move the sexual and reproductive health and rights agenda forward. Three suggested areas for further work follow.

Improvement of Measurement and Accountability

Measurement and accountability issues play multifaceted and increasingly important roles in global health.^{16,17} There are 3 areas related to measurement where sexual and reproductive health and rights require improvement and attention. The first is the refining of methods for collecting evidence to better document the problem of sexual and reproductive health and rights, both in terms of health and more broadly. A multilevel evaluation approach is required to measure the enabling environment for sexual and reproductive health and rights—including the social, economic, and political determinants

of reproductive health and the measurement of intergenerational impacts.¹⁸ In a similar vein, sexual and reproductive health and rights programs require the development of meaningful indicators that measure the importance of human rights in addressing sexual and reproductive health and rights. Better evidence demonstrating the links between reproductive health and poverty reduction is also critical to ensuring that sexual and reproductive health and rights stay engaged with the international policy community.

The second area for improvement is the indicators that measure the success of sexual and reproductive health policies and programs. Now more than ever, the sexual and reproductive health and rights field must be able to show measurable results of policies and programs to donors and policymakers alike. This requires differentiation between policy and program indicators. Local experience can be used to bolster conceptually sound and well-developed arguments about why investments in sexual and reproductive health and rights make a difference.

A third area related to measurement is improvement of the tracking of resource flows for reproductive health, particularly at the national and subnational levels. This will improve the accountability of both donors and countries receiving development assistance. It will also ultimately strengthen the evidence base related to the cost-effectiveness of sexual and reproductive health and rights interventions. To build this evidence base, it is necessary for sexual and reproductive health and rights advocates to communicate better with economists, many of whom are making the decisions about where health resources go.

Creation and Renewal of Alliances for Advocacy

Ensuring that the global reproductive health agenda is on secure footing also requires an understanding of how sexual and reproductive health and rights is positioned with respect to other agendas and the ability to forge alliances with key partners. A first step is to more actively engage sexual and reproductive health and rights advocates and practitioners at the domestic and international levels. Regional and national women's groups play the critical role of providing the evidence of sexual and reproductive health and rights problems and the effectiveness of their interventions.¹⁹ Ensuring that this evidence is heard at the international level and among a range of stakeholders is an important function for national women's groups and requires better coordination between women's groups at all levels.

An additional step in alliance building is to reextend a hand to those from the population and family-planning fields who helped to create consensus at ICPD. SHRH are the result of the merging of several networks, including family planning and population²⁰; revisiting these alliances that built ICPD can strengthen the current status of sexual and reproductive health and rights.

A third step in alliance building is for the sexual and reproductive health and rights field to engage more vigorously with stakeholders in related networks close at hand, particularly HIV/AIDS and global health policy. There are obvious connections between sexual and reproductive health and rights and HIV/AIDS. In practice, however, the maintenance of linkages between the sexual and reproductive health

and rights and HIV/AIDS communities has been fraught with difficulties.²¹ Conflicts over limited resources and concerns about divergent advocacy agendas have created unease. As HIV has become feminized, there are increasing calls to join forces.²² Similarly, sexual and reproductive health and rights could forge alliances with another key network that is strengthening the global health workforce and health systems in general. Clearly, everyone in the health sector is better served by supporting functional and appropriately staffed health services; an alliance between safe motherhood advocates and emergency first responders, for example, makes for a creative alignment of bedfellows that is likely to accomplish more than remaining within narrowly defined communities.

Finally, we need to reach out to networks where there may be less immediately common ground. The transnational networks that converged in 1994 to form the skeleton for ICPD need to be knitted together anew. The sexual and reproductive health and rights field must create room for more actors—particularly development planners and those who are actively engaged with the planning and budgeting process.¹⁴ On the basis of the experience of the past decade, we must be prepared to engage with faith-based groups and their leaders, with the recognition that culture and faith influence individual women's and men's decisions about sexuality and reproduction in profound ways.

New Strategies for Mobilization of Resources

The sexual and reproductive health and rights community

operates in an increasingly complicated funding and donor assistance world. On the one hand, there has been a move toward vertical single-disease global health initiatives,²³ while on the other hand, countries are encouraged to move toward sector-wide approaches and general budget support. Much of the sexual and reproductive health and rights agenda is left out of the vertical single-disease funding approach, and much gets lost in the World Bank-supported Poverty Reduction Strategy Papers. This suggests the need for a 2-pronged sexual and reproductive health and rights resource mobilization strategy that taps these divergent funding mechanisms.

The field of sexual and reproductive health and rights is dynamic, with a landscape that has changed and will continue to change. What the Cairo conference got right—that economic and technological development requires governments and other appropriate parties to ensure that individuals can make meaningful and informed choices related to their sexual and reproductive lives—was not unsubstantial. Empowerment through education and legal and policy reforms, as well as the provision of a range of health services and information related to reproduction and sexuality, comprise many of the action points in the Program of Action. Now is the time to move the sexual and reproductive health and rights agenda forward—by recognizing and celebrating the enduring value of ICPD's legacy. ■

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